

AUTONOMIC AND SYNCOPE CLINIC CONSULTATION

Date of service: 2026-03-18

Patient: Elena Marquez

Page 1 of 2 - History and physiologic assessment

REFERRAL QUESTION

Elena was referred after recurrent episodes of dimming vision and leg weakness in upright, warm environments. The consultation asks whether these events represent reflex syncope, orthostatic hypotension, a rhythm disturbance, a vestibular disorder, or another neurologic cause. The records reviewed include an emergency note, orthostatic vital signs, a cardiology report, a head CT report, and a separate vestibular evaluation.

EVENT HISTORY

The earliest event in the available history occurred after several days of reduced fluid intake. Elena rose from a kitchen chair, walked toward the sink, and developed narrowing vision with a sense that she might black out. She held the counter and was helped to a chair. She remained responsive and remembered the event. More recent events have followed prolonged standing while cooking, waiting in a pharmacy line, or talking with neighbors outdoors in warm weather. Sitting or lying down generally improves symptoms within two to five minutes.

Elena uses precise language when comparing symptoms. During upright episodes, the dominant sensation is gray vision, muffled hearing, warmth, and loss of strength rather than rotation of the room. During a different set of brief events triggered by rolling in bed, she experiences spinning without impending loss of consciousness. Those positional spinning events improved after canalith repositioning. The two symptom groups can occur in the same month but have different triggers and associated features.

She denies sudden collapse without warning, exertional chest pain, a sustained racing heartbeat, tongue injury, prolonged confusion, or witnessed tonic-clonic movement. There is no family history of unexplained sudden death. She has never lost bladder control during an event. Her daughter has seen Elena become pale and quiet while standing but has not seen unresponsiveness. Elena has learned to sit at the first sign of visual dimming.

MEDICATION AND INTAKE REVIEW

Current medications include lisinopril, hydrochlorothiazide, metformin, and a nightly statin. She takes the blood-pressure medicines in the morning. On busy days she may drink only two cups of coffee and one glass of water before late afternoon. Breakfast is sometimes skipped. She does not use an alpha blocker, nitrate, tricyclic antidepressant, or sedative. There has been no recent gastrointestinal fluid loss. Home blood-pressure readings are usually in the 120s to 130s systolic while seated.

EXAMINATION

Elena was comfortable and conversant while supine. Cardiac rhythm was regular, lungs were clear, and there was no lower-extremity edema. Speech, eye movements, facial symmetry, strength, sensation, and coordination were normal. Supine blood pressure after ten minutes of rest was 138/74 mmHg with heart rate 72. At one minute standing, blood pressure was 112/68 with heart rate 88. At three minutes standing, blood pressure was 104/66 with heart rate 96. She reported gray vision and heaviness in both legs but no spinning. There was no focal neurologic change.

An abdominal binder was applied and the active stand was repeated after recovery. The systolic pressure decline was smaller, and Elena remained comfortable through five minutes. This response does not establish a single cause by itself, but it supports a clinically meaningful orthostatic component. Carotid sinus massage was not performed. The prior ambulatory rhythm report did not identify sustained tachyarrhythmia or a clinically significant pause during patient-triggered recordings.

PAGE 1 IMPRESSION

The history and observed blood-pressure decline support orthostatic intolerance with presyncopal symptoms. The reproducible brief spinning treated in vestibular clinic is a separate syndrome. Page 2 documents the most recent heat-triggered event, the medication decision, and the complete safety plan.

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Page 2 of 2 - Trigger analysis, assessment, and plan

MOST RECENT EVENT

The most informative recent episode occurred immediately after a hot shower. Elena had taken lisinopril and hydrochlorothiazide that morning and had eaten only toast. The bathroom was warm and steamy. While standing to dry her hair, she noticed ringing in both ears, gray vision, nausea, and a rapid loss of strength in her legs. She sat on the closed toilet lid before falling. Her daughter brought water and helped her lie down with her legs elevated. Symptoms resolved over approximately four minutes. There was no room-spinning sensation, chest pain, sustained palpitation, loss of consciousness, injury, confusion, or focal weakness.

TRIGGER INTERPRETATION

Heat causes peripheral vasodilation, standing shifts blood toward the lower body, and limited morning intake reduces the volume available to maintain pressure. A thiazide diuretic can increase susceptibility in that setting. The event therefore fits the same orthostatic presyncope pattern observed during active stand testing. It does not resemble the brief right-sided positional vertigo documented in the neurology note, and it does not have the abrupt unheralded character expected from some malignant rhythm disorders.

ASSESSMENT

1. Symptomatic orthostatic hypotension with presyncope, demonstrated by a systolic blood-pressure decline from 138 mmHg supine to 104 mmHg after three minutes standing with reproduction of gray vision and bilateral leg heaviness.
2. Heat, prolonged standing, low fluid intake, and morning diuretic exposure are modifiable amplifiers.
3. No documented complete loss of consciousness. No current evidence of sustained arrhythmia, seizure, acute stroke, or a continuous central vestibular syndrome.
4. Separately treated right posterior canal benign paroxysmal positional vertigo, clinically distinct from the upright presyncopal events.

MEDICATION PLAN

Hydrochlorothiazide will be stopped today after discussion with Elena and her primary-care clinician. Lisinopril will continue at the current dose for now. Elena will record seated and standing blood pressure each morning and whenever symptoms occur. The primary-care team will review the log within two weeks and decide whether additional antihypertensive adjustment is needed. She was instructed not to restart the diuretic on her own, even if a single seated reading is elevated.

NONPHARMACOLOGIC PLAN

Elena will aim for approximately two liters of nonalcoholic fluid daily unless another clinician identifies a contraindication. She will eat regular meals, avoid entering a hot shower when fasting, use warm rather than very hot water, and sit while drying or dressing if symptoms begin. Slow transitions, calf-pumping before standing, and brief counter-pressure maneuvers were demonstrated. Waist-high compression garments or the abdominal binder may be used for prolonged upright activity. A shower chair and nonslip surface were recommended to reduce injury risk.

SAFETY AND FOLLOW-UP

If warning symptoms begin, Elena should sit or lie down immediately rather than trying to walk to another room. Emergency evaluation is appropriate for actual loss of consciousness during exertion or while supine, a significant injury, chest pain, persistent rapid heartbeat, new neurologic deficit, black or bloody stool, or symptoms that do not improve promptly after lying down. Driving should be avoided if events occur without enough warning to stop safely.

A nurse will call in one week to review symptoms and the blood-pressure log. Formal follow-up is planned in four weeks. If events continue despite removing hydrochlorothiazide and improving hydration, the next steps may include longer rhythm monitoring and standardized autonomic testing. Treatment decisions will be based on documented physiology and clinical response rather than on semantic-search score.

PAGE 2 SUMMARY

The decisive details are that the latest near-fainting episode followed a hot shower, involved gray vision without spinning or loss of consciousness, and led to a plan to stop hydrochlorothiazide. These details are intentionally located on page 2 so a successful PDF semantic search can demonstrate page-specific provenance as Binary.data#page=2.

SYNTHETIC DEMO DOCUMENT

This fictional consultation is intended only to test text extraction, chunking, semantic retrieval, and provenance from a two-page PDF Binary. It is not medical advice and does not describe a real person.